CLINICAL TRIAL FAILURES · DISCONTINUATION RATE · ONCOLOGY PHASE 2,3 · STARTS 2015–2024

MET: 8 molecules stopped early

10 stopped trials across 8 molecules: Cabozantinib, Amg-337, Amivantamab, Anti-EGFR/c-Met Antibody-drug Conjugate GEN1286 and 4 more. 6 of the stops were the trial's own verdict and 2 followed a decision taken elsewhere; 2 cannot be established. Against 5.2% for all oncology trials with a resolved drug target and 4.5% across all oncology Phase 2,3 trials in the same window. The rate counts trials that stopped early for efficacy, safety or benefit–risk reasons. Trials that ran to completion and missed their primary endpoint are counted separately below, never in the rate.

8.1%
10 of 123 closed trials (95% CI 4.5%–14.3%)

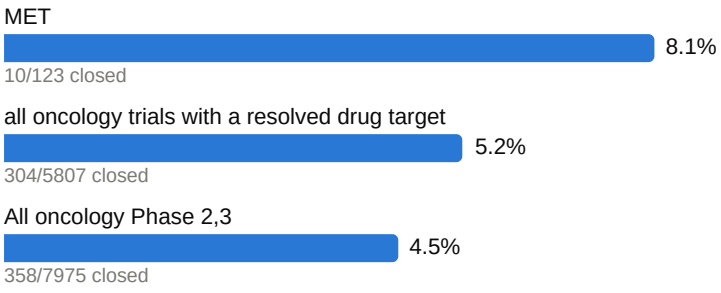
6 / 3 / 0
efficacy / safety / both (adds to 10)

38.7%
of 318 trials have closed · 195 still open or unresolved

2.3%
stopped for a biological reason within 3 years of starting (95% CI 1.0%–5.0%; 188 still at risk; 1.5% area-wide)

2 of 3 **Completed and missed the primary endpoint.** Of 74 completed trials in this segment, 3 posted a primary result on ClinicalTrials.gov that can be read — the sponsor's own comparison held to the sponsor's own threshold, or its statement that the endpoint was missed: 2 missed and 1 met. Counted separately and never in the rate above; a floor, not a rate. 30 of the 67 completed trials that finished more than a year ago have posted no results at all — a gap in what can be known, not a sign of failure.

How this compares



What is in this class: 213 distinct experimental drugs across 318 trials (small molecule 295, antibody 134, unknown 21, adc 12). Most tested: CABOZANTINIB (134), NIVOLUMAB (40), CRIZOTINIB (34), CAPMATINIB (28), AMIVANTAMAB (28). A class groups drugs by what they act on; check that the grouping is one you would make before reading the rate as a property of the mechanism.

How much does this rest on one decision? The 10 stops came from 10 sponsor-asset programmes across 10 sponsors; the largest (University of Nebraska) contributed 1; removing that programme's trials from both sides leaves 7.4%. Screened alongside 85 other segments, the rate is within what screening that many classes could produce (q=1); the molecules and trials that failed here are the record either way. A further 5 closed trials here were terminated with no cause recorded in the registry; if every one of them were biological the rate would be 12.2%. Read it as a screen worth checking against the underlying trials.

Bars show the share of closed trials stopped for biological reasons. Confidence intervals overlap where sample sizes are small — read the counts, not just the bars.

By start cohort

Trials started	Stops	Closed	Rate	95% CI
2015–2017	2	42	4.8%	1.3%–15.8%
2018–2020	6	45	13.3%	6.3%–26.2%
2021–2024	2	36	5.6%	1.5%–18.1%

Where the stops sit

Efficacy / futility: 6, Safety: 3, Benefit–risk: 1.
Sponsors with most stops: Genmab (1), Johnson & Johnson (1), Jiangsu Alphamab Biopharmaceuticals Co., Ltd (1), Novartis (1), NantPharma, LLC (1), University of Nebraska (1).

The stopped trials

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT06685068	2	Genmab GMAB	Anti-EGFR/c-Met Antibody-drug Con…	2026-03	Benefit–risk	Genmab has decided to terminate the GEN1286 program due to an unfavorable benefit-risk profile not supporting further development.

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT05653427	2	Johnson & Johnson JNJ	AMIVANTAMAB	2023-10	Efficacy	N\~15 subjects data analysis did not meet statistical consideration of ORR and study primary/secondary objectives were not met. Enrollment was terminated for futility
NCT03838848	2	Jiangsu Alphamab Biopharmaceuticals Co., Ltd	ERFONRILIMAB, CARBOPLATIN, PEMETREXED, NINGETINIB	2022-07	Efficacy	Cohort A,B,C end enrollments. Cohort D and E were considered to have no significant clinical benefit at the SMC meeting and decided to terminate enrollment.
NCT04139317	2	Novartis NVS	CAPMATINIB	2022-01	Safety	Sponsor decision to terminate due to the lack of tolerability observed in patients treated with capmatinib and pembrolizumab in the combination arm as compared to patients treated with pembrolizumab alone in the pembrolizumab single agent arm
NCT03132155	2	NantPharma, LLC	AMG-337	2021-08	Efficacy	Prematurely terminated due to lack of therapeutic effect
NCT03201250	2	University of Nebraska	CABOZANTINIB	2021-04	Efficacy	Phase I response to treatment was not as expected and the myeloma field in treatment has changed since initiating the study, so investigators felt the study should not be pursued into Phase II.

The 6 most recent of 10 stopped trials. Every molecule behind all 10 is named above; the complete trial list, the trials still running and the terminations with no readable cause ship with the dataset and the diligence report.

Method

Denominator: ClinicalTrials.gov interventional Phase 2,3 oncology trials started 2015–2024 that have closed (completed or terminated). Numerator: terminated trials whose registry stop reason is classified as biological (efficacy, safety or benefit–risk) by Classification V2 — held-out precision 95.5%, recall 95.3% (n=600). Drugs are linked to ChEMBL and the NCI Thesaurus; 77% of industry oncology trials in this window carry a resolved drug target, and a trial without one cannot enter a mechanism class. Intervals are Wilson 95%.

Limits

Not a failure rate: completed trials that missed their endpoints are not in the rate. They are counted separately where the sponsor posted a primary result that can be read — most did not, so that count is a floor — and programmes discontinued after a completed trial do not appear. Stop reasons are sponsor-reported. This is a closed-trial proportion, not a time-to-event analysis: only 38.7% of trials in this segment have closed, and a trial that stops early enters the denominator sooner than one that runs to completion, which can inflate the rate in immature segments. Recent cohorts have fewer closed trials, so their rates are less stable. Research signals, not clinical or investment advice.

The whole cohort, trial by trial. The diligence report names every trial behind these figures — the ones still running and the ones that completed and missed their primary endpoint included — and places your molecule against each one that failed. Free during the beta: enter your work email at **clinicaltrialfailures.com/asset-check** and it opens straight away.

Sources: ClinicalTrials.gov (NLM); ChEMBL (EMBL-EBI, CC BY-SA 3.0); NCI Thesaurus (NCI); RxNorm/RxClass (NLM); SEC EDGAR. Classification, linkage and rates by Clinical Trial Failures. Rebuilt weekly; this brief covers trials started 2015–2024.